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OCT 24 2019

Anabel Renteria
Initiative Coordinator
Office of the Attorney General
P.O. Box 944255
Sacramento, CA 94244-2550

INITIATIVE COORDINATOR
ATTORNEY GENERAL'S OFFICE

Re: **Request for Title and Summary for Proposed Initiative**

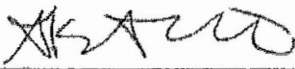
Dear Ms. Renteria:

Pursuant to Article II, Section 10(d) of the California Constitution, I submit the attached proposed Initiative, entitled "Protect the Lives of Dialysis Patients Act," to your office and request that your office prepare a title and summary. Included with this submission is the required proponent certifications pursuant to sections 9001 and 9608 of the California Elections Code, along with a check for \$2,000.00.

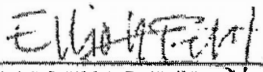
All inquiries or correspondence relative to this initiative should be directed to George Yin at Kaufman Legal Group, APC, 777 S. Figueroa St., Suite 4050, Los Angeles, CA 90017.

Thank you for your assistance.

Very truly yours,



SARAH STECK



ELLIOTT PETTY

This initiative measure is submitted to the people in accordance with the provisions of Article II, Section 8, of the California Constitution.

This initiative measure amends and adds sections to the Health and Safety Code; therefore, existing provisions proposed to be deleted are printed in ~~strikeout~~ type and new provisions proposed to be added are printed in *italic* type to indicate that they are new.

SEC. 1. Name

This act shall be known as the “Protect the Lives of Dialysis Patients Act.”

SEC. 2. Findings and Purposes

This act, adopted by the people of the State of California, makes the following Findings and has the following Purposes:

A. The People make the following findings:

(1) Kidney dialysis is a life-saving process in which blood is removed from a person’s body, cleaned of toxins, and then returned to the patient. Generally, it must be done at least three times a week for several hours a session, and the patient must continue treatment for the rest of their lives or until they can obtain a kidney transplant.

(2) In California, at least 68,000 Californians undergo dialysis treatment.

(3) Just two multinational, for-profit corporations operate or manage nearly three-quarters of dialysis clinics in California and treat almost 70 percent of dialysis patients in the state. These two multinational corporations annually earn billions of dollars from their dialysis operations, including more than \$360 million each year in California alone.

(4) The dialysis procedure and side effects from the treatments present several dangers to patients, and many dialysis clinics in California have been cited for failure to maintain proper standards of care. Failure to maintain proper standards can lead to patient harm, hospitalizations, and even death.

(5) Dialysis clinics are currently not required to maintain a doctor on site to oversee quality, ensure the patient plan of care is appropriately followed, and monitor safety protocols.

(6) Dialysis treatments involve direct access to the bloodstream, which puts patients at heightened risk of getting dangerous infections. Proper reporting and transparency of infection rates encourages clinics to improve quality and help patients make the best choice for their care.

(7) When health care facilities like hospitals and nursing homes close, California regulators are able to take steps to protect patients from harm. Equivalent protections should be provided to vulnerable patients when dialysis clinics close.

(9) Dialysis corporations have lobbied against efforts to enact protections for kidney dialysis patients in California, spending over \$100 million in 2018 and 2019 to influence California voters and the Legislature.

B. Purposes:

(1) It is the purpose of this Act to ensure that outpatient kidney dialysis clinics provide quality and affordable patient care to people suffering from end stage renal disease.

(2) This Act is intended to be budget neutral for the State to implement and administer.

SEC. 3. Section 1226.7 is added to the Health and Safety Code, to read:

1226.7 (a) A chronic dialysis clinic shall not discriminate with respect to offering or providing care, and shall not refuse to offer or to provide care, on the basis of who is responsible for paying for a patient's treatment. Such prohibited discrimination includes, but is not limited to, discrimination on the basis that a payer is an individual patient, private entity, insurer, Medi-Cal, Medicaid, or Medicare.

(b) A chronic dialysis clinic shall not terminate, abridge, modify, or fail to perform under any agreement to provide services to patients covered by Medi-Cal, Medicaid, or Medicare on the basis of the requirements imposed by this chapter.

SEC. 4. Section 1226.8 is added to the Health and Safety Code, to read:

1226.8. (a) Every chronic dialysis clinic must maintain at minimum one licensed physician present on site during all operating hours. This physician shall be accountable for the quality of medical care that the chronic dialysis clinic provides to all patients on site.

(1) A chronic dialysis clinic may apply to the department for an exception to the requirement in subdivision (a) on the grounds that a bona fide shortage of qualified physicians prevents it from satisfying the requirement. Upon such a showing, the department may grant an exception that permits the clinic to satisfy the requirement in subdivision (a) by having at minimum one of the following on site during all operating hours: a licensed physician, a nurse practitioner, or a physician assistant, who shall be accountable as described in subdivision (a).

(2) The duration of an exception granted by the department pursuant to paragraph (1) shall be one calendar year from the date the clinic is notified of the department's determination.

(b) For each chronic dialysis clinic, the clinic or its governing entity shall quarterly report to the department, on a form and schedule prescribed by the department, dialysis clinic health care associated infection ("dialysis clinic HAI") data, including the incidence of dialysis clinic HAIs at each chronic dialysis clinic in California. The chief executive officer or other principal officer of the clinic or governing entity shall certify under penalty of perjury that he or she is satisfied, after review, that the dialysis clinic HAI report submitted to the department is accurate and complete. The department shall post on its Web site the dialysis clinic HAI data. The posted information shall include information identifying the governing entity of each chronic dialysis clinic.

(1) In addition to reporting to the department pursuant to the requirements of this section, chronic dialysis clinics shall report dialysis clinic HAI data to the National Healthcare Safety Network in accordance with National Healthcare Safety Network requirements and procedures.

(c) In the event the department determines that a chronic dialysis clinic or governing entity failed to maintain the information or timely submit a report required under this section, or that the report submitted was inaccurate or incomplete, the department shall assess a penalty against the chronic dialysis clinic or governing entity not to exceed one hundred thousand dollars (\$100,000). The department shall determine the amount of the penalty based on the severity of the violation, the materiality of the inaccuracy or omitted information, and the strength of the explanation, if any, for the violation. Penalties collected pursuant to this paragraph shall be used by the department to implement and enforce laws governing chronic dialysis clinics. If a chronic dialysis clinic or governing entity disputes a determination by the department to assess a penalty pursuant to this section, or the amount of an administrative penalty, the chronic dialysis clinic or governing entity may, within 10 working days, request a hearing pursuant to Section 131071. A chronic dialysis clinic or governing entity shall promptly pay all administrative penalties when all appeals have been exhausted and the department's position has been upheld.

(d) Definitions

For purposes of this section:

(1) "Dialysis clinic HAI" means a bloodstream infection, local access site infection, or vascular access infection related to a dialysis event as defined by the National Health and Safety Network of the federal Centers for Disease Control and Prevention, or any appropriate additional or alternative definition that the department defines by regulation.

(2) "Governing entity" has the same meaning as in Section 1226.7.

(3) "Licensed physician" means a nephrologist or other physician licensed by the state pursuant to Chapter 5 of Division 2 of the Business and Professions Code.

(3) "National Healthcare Safety Network" or "NHSN" means the secure, Internet-based system developed and managed by the federal Centers for Disease Control and Prevention (CDC) that collects, analyzes, and reports risk-adjusted Dialysis center HAI data related to the incidence of HAI and the process measures implemented to prevent these infections, or any successor data collection system that serves substantially the same purpose.

(4) "Nurse practitioner" means a registered nurse licensed pursuant to Chapter 6 of Division 2 of the Business and Professions Code and certified as a nurse practitioner by the Board of Registered Nursing.

(5) "Physician assistant" means a physician assistant licensed pursuant to Chapter 7.7 of Division 2 of the Business and Professions Code.

SECTION 5. Section 1226.9 is added to the Health and Safety Code, to read:

1226.9. (a) Prior to closing a chronic dialysis clinic, or substantially reducing or eliminating the level of services provided by a chronic dialysis clinic, the clinic or its governing entity must provide written notice to, and obtain the written consent of, the department.

(b) The department shall have discretion to consent to, give conditional consent to, or not consent to, any proposed closure or substantial reduction or elimination of services. In making its determination, the department shall consider any factors that the department considers relevant, including, but not limited to, the following:

(1) The effect on the availability and accessibility of health care services to the affected community, including the clinic's detailed plan for ensuring patients will have uninterrupted access to care.

(2) Evidence of good faith efforts by the clinic or governing entity to sell, lease, or otherwise transfer ownership or operations of the clinic to another entity that would provide chronic dialysis care.

(c) The department shall adopt regulations implementing this section.

(d) Definitions:

(1) "Governing entity" has the same meaning as in Section 1226.7.

SEC. 6. Section 1266.3 is added to the Health and Safety Code, to read:

1266.3. It is the intent of the People that California taxpayers not be financially responsible for implementation and enforcement of the Protect the Lives of Dialysis Patients Act. In order to effectuate that intent, when calculating, assessing, and collecting fees imposed on chronic dialysis clinics pursuant to Section 1266, the department shall take into account all costs associated with implementing and enforcing Sections 1226.7 through 1226.9.

SEC. 7. Nothing in this act is intended to affect health facilities licensed pursuant to subdivision (a), (b), or (f) of Section 1250 of the Health and Safety Code.

SEC. 8. The State Department of Public Health shall adopt emergency regulations to implement this act no later than one year following its effective date. The adoption of emergency regulations implementing this act shall be deemed an emergency and necessary for the immediate preservation of the public peace, health, safety, or general welfare.

SEC. 9. Pursuant to subdivision (c) of Section 10 of Article II of the California Constitution, this Act may be amended either (1) by a subsequent measure submitted to a vote of the people at a statewide election; or (2) by a statute validly passed by the Legislature and signed by the Governor, but only to further the purposes of the Act.

SEC. 10. The provisions of this act are severable. If any provision of this act or its application is held invalid, that invalidity shall not affect other provisions or applications that can be given effect without the invalid provision or application.